

CNOR AT A GLANCE

Owner	CCI — Competency & Credentialing Institute
Items	200 multiple choice — 185 scored, 15 pretest
Time	3 hours 45 minutes
Scoring	Scaled 200–800; 620 or higher passes
Standard setting	Modified Angoff, set by CNOR-certified experts
Benchmark	What an RN with 2 years and 2,400 perioperative hours should know
Eligibility	Current unrestricted RN licence and current perioperative work; 2 years and 2,400 perioperative hours with at least 1,200 intraoperative — or 18 months and 2,400 hours for CFPN, CST, TS-C or military-equivalent holders
Accreditation	NCCA and ABSNC accredited

SUBJECT AREAS ON THE EXAM

Subject area	Weight	Items
Pre/postoperative patient assessment and diagnosis	15%	28
Individualized plan of care and expected outcomes	8%	15
Intraoperative: patient care and safety	25%	46
Intraoperative: personnel, services and materials	9%	17
Communication and documentation	11%	20
Infection prevention and control of environment, instruments, supplies	16%	30
Emergency situations	10%	19
Professional accountabilities	6%	10

TIP The two intraoperative areas plus infection prevention are half the exam. Spend your study time there before professional accountability.

STERILE FIELD RULES

- Only sterile items touch sterile items; if in doubt, consider it contaminated.
- Package edges and the outer margin of a drape are not sterile.
- A sterile table is sterile at table level only — nothing below the edge comes back.
- Gown fronts are sterile from chest to field level; sleeves are sterile from about two inches above the elbow to the cuff, and the cuffs stay covered by sterile gloves.
- Keep the field in view at all times; never turn your back to it.
- Move face to face or back to back; pass with the sterile side toward the field.
- Moisture wicks bacteria — a wet drape is a contaminated drape.

UNIVERSAL PROTOCOL

1. Verify the patient, procedure, site and consent before the procedure.
2. Mark the site with the patient involved and awake wherever possible.
3. Time out immediately before incision, with everyone stopped and participating.
4. Any unresolved discrepancy stops the procedure until it is settled.

COUNTS

- Establish baseline counts for sponges and sharps, and for instruments when a body cavity is opened, per policy.
- Count again before closure of a cavity within a cavity, before wound closure begins, and at skin closure or the end of the procedure.
- Count when either the scrub person or the circulating nurse is relieved.
- Nothing counted leaves the room until the final count is reconciled.
- An incorrect count requires recount/search and immediate team notification; if the missing item is not recovered, obtain intraoperative imaging before final closure when the patient's condition permits, then document the discrepancy and all reconciliation actions.



STERILISATION QUICK REFERENCE

Element	Remember
Steam, gravity	About 121 °C (250 °F); longer exposure
Steam, dynamic air removal	About 132–135 °C (270–275 °F); short exposure
Always	Follow the device manufacturer's instructions for use
Biological indicator	Directly monitors killing power with resistant spores; read it with mechanical and chemical monitoring — a negative result does not prove every item is sterile
Bowie-Dick test	Daily air-removal check on a dynamic-air-removal steriliser
Chemical indicators	External shows processed; internal shows conditions inside
Low-temperature methods	For heat- or moisture-sensitive devices
Immediate-use steam	For urgent need, not convenience; not for implants except a documented emergency with no alternative and full monitoring

ASSESSMENT, PLAN OF CARE, DOCUMENTATION

- Verify consent, allergies, fasting status, implants and anticoagulants preoperatively.
- Flag latex allergy, difficult airway, malignant hyperthermia history and mobility limits.
- Build the plan of care around measurable expected outcomes, not tasks.
- Hand off to PACU with procedure, anaesthesia, lines, drains, counts and concerns.
- Document at the time, in facts: who, what, when, and the patient's response.
- Never chart in advance, and never alter an entry — amend it.

INFECTION PREVENTION AND ENVIRONMENT

- Three zones: unrestricted, semi-restricted, restricted; attire follows the zone.
- Positive pressure, controlled air exchanges, and doors kept closed.
- Limit traffic and talking over an open field.
- Point-of-use treatment keeps bioburden from drying on instruments.
- Clean and decontaminate before any sterilisation — soil blocks sterilant.
- Surgical smoke is a hazard: evacuate at the source.

PATIENT SAFETY IN THE ROOM

- Position to protect skin, nerves and pressure points; document who positioned.
- Supine risks ulnar and brachial plexus injury; lithotomy risks peroneal and femoral.
- Prone: keep eyes free of pressure and protect breasts and genitalia. Lateral: place the axillary roll just caudal to the dependent axilla, not in it.
- Dispersive electrode on clean, dry, well-perfused muscle, close to the site.
- Fire triangle: ignition source, fuel, oxidiser — let alcohol prep dry fully.
- Prevent unplanned hypothermia with active warming and warmed fluids.
- Label every specimen at the field and read it back before it leaves the room.

EMERGENCIES

- Malignant hyperthermia: an unexplained rise in end-tidal CO₂ is an important early sign; stop triggers, call for the MH cart and give dantrolene.
- Anaphylaxis: stop the suspected trigger, call for help, give epinephrine promptly and support airway, oxygenation and circulation; common triggers are antibiotics, neuromuscular blockers and latex.
- Haemorrhage: call for help, blood products and rapid infusion; assign one recorder.
- Cardiac arrest: defibrillator and crash cart location must be known before the shift.
- Every emergency ends with a debrief and factual documentation.

TEST-DAY PLAN

1. About 67 seconds per item — flag and move rather than stall.
2. Choose the answer that protects the patient and the sterile field first.
3. When two answers are safe, pick the one done before, not after, the incision.
4. Prefer stopping and verifying over proceeding to save time.
5. Answer every item; there is no penalty for a guess.

TIP Study aid only — not clinical guidance. Sterilisation parameters follow the device and steriliser instructions for use; confirm exam details in the current CCI CNOR Candidate Handbook.

